Sample Referral Letters

200 synthetic referrals for testing. All practices, patients and clinical details are fictional. Roughly a quarter are deliberately incomplete; the answer key at the end lists which and why.

Referral #001

```
REFERRING PRACTICE: PRAC-BRK-635
PRIORITY: URGENT
PATIENT: TEST-299982

47-year-old woman with a swollen, hot, painful right calf for three days.
Long-haul flight last week. On combined pill. Wells score 3. D-dimer raised.
Started on apixaban treatment dose pending scan. Advised 999 for chest pain or
breathlessness.
```

Referral #002

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REFERRING PRACTICE: PRAC-OAK-196
PRIORITY: ROUTINE
PATIENT: TEST-361032

Thank you for seeing this patient.
```

Referral #003

```
REFERRING PRACTICE: PRAC-HGT-251
PRIORITY: URGENT
PATIENT: TEST-541254

69-year-old man with unintentional weight loss of 8 kg over three months and new
change in bowel habit to looser stools. Iron-deficiency anaemia picked up on
routine bloods; no prior GI history. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin
6. Abdominal exam unremarkable. Advised to attend ED with any rectal bleeding
causing dizziness or severe abdominal pain.

CLINICIAN NOTE: FIT positive above threshold.
```

Referral #004

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REFERRING PRACTICE: PRAC-BRK-946
PRIORITY: ROUTINE
PATIENT: TEST-394405

52-year-old woman with gallstones on ultrasound and three episodes of biliary
colic in four months. One A&E attendance last month; discharged with analgesia.
LFTs normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised
to attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #005

```
REFERRING PRACTICE: PRAC-RVS-229
PRIORITY: URGENT
PATIENT: TEST-404718

67-year-old woman with three weeks of painless visible haematuria, two episodes.
Previous smoker, quit 2015. No prior urological history. Urine dip positive for
blood, MSU negative for infection. U&E normal, eGFR 74. Advised to attend ED if
unable to pass urine or passing clots.

CLINICIAN NOTE: Meets 2-week-wait criteria for suspected bladder cancer.
```

Referral #006

```
REFERRING PRACTICE: PRAC-MPL-314
PRIORITY: ROUTINE
PATIENT: TEST-159105

As per phone call.
```

Referral #007

```
REFERRING PRACTICE: PRAC-WLW-683
PRIORITY: URGENT
PATIENT: TEST-519838

Please assess and advise. Many thanks.
```

Referral #008

```
REFERRING PRACTICE: PRAC-OAK-678
PRIORITY: ROUTINE
PATIENT: TEST-430349

Please assess and advise. Many thanks.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #009

```
REFERRING PRACTICE: PRAC-CLV-921
PRIORITY: URGENT
PATIENT: TEST-415516

55-year-old woman with sudden painless loss of vision in the right eye this
morning, now partially recovered. Hypertension, type 2 diabetes, smoker. Visual
acuity 6/12 right, 6/6 left. ECG sinus rhythm. Started aspirin 300 mg. Advised 999
if any weakness, speech disturbance or further visual loss.

CLINICIAN NOTE: Possible amaurosis fugax - TIA pathway may be more appropriate if
not accepted.
```

Referral #010

```
REFERRING PRACTICE: PRAC-OAK-949
PRIORITY: ROUTINE
PATIENT: TEST-326015

69-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Hypothyroid on levothyroxine. Works on a
production line. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #011

```
REFERRING PRACTICE: PRAC-OAK-217
PRIORITY: ROUTINE
PATIENT: TEST-757920

77-year-old man with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #012

```
REFERRING PRACTICE: PRAC-BRK-150
PRIORITY: ROUTINE
PATIENT: TEST-336962

29-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #013

```
REFERRING PRACTICE: PRAC-RVS-857
PRIORITY: URGENT
PATIENT: TEST-245262

52-year-old woman with sudden painless loss of vision in the right eye this
morning, now partially recovered. No previous eye problems. Takes no regular
medication. Visual acuity 6/12 right, 6/6 left. ECG sinus rhythm. Started aspirin
300 mg. Advised 999 if any weakness, speech disturbance or further visual loss.

CLINICIAN NOTE: Possible amaurosis fugax - TIA pathway may be more appropriate if
not accepted.
```

Referral #014

```
REFERRING PRACTICE: PRAC-OAK-729
PRIORITY: URGENT
PATIENT: TEST-942047

77-year-old woman with post-menopausal bleeding, two episodes over the past month.
Menopause age 51. Not on HRT. BMI 38. Speculum: cervix looks normal. Smear up to
date and normal. Told to expect contact within two weeks.
```

Referral #015

```
REFERRING PRACTICE: PRAC-BRK-872
PRIORITY: ROUTINE
PATIENT: TEST-828297

73-year-old woman with recurrent heartburn and regurgitation despite eight weeks
of full-dose omeprazole. Hiatus hernia suspected on previous barium swallow
(2019). FBC normal. Lifestyle advice given; trial of alginate added. Told to seek
review promptly if difficulty swallowing, vomiting blood or black stools.
```

Referral #016

```
REFERRING PRACTICE: PRAC-OAK-461
PATIENT: TEST-896431

84-year-old woman with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #017

```
REFERRING PRACTICE: PRAC-WLW-517
PATIENT: TEST-217187

57-year-old woman with lower back pain radiating to the left leg for 14 weeks. Previous episode 5 years ago settled with physio. Straight leg raise positive at 40 degrees. Power and reflexes normal. No red flags. Cauda equina safety-netting given verbally and in writing.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #018

```
REFERRING PRACTICE: PRAC-OAK-261
PRIORITY: URGENT
PATIENT: TEST-970540

68-year-old woman with a persistent hoarse voice for four weeks. Current smoker, 20 per day. Drinks around 30 units a week. No neck lumps palpable. Chest X-ray requested, result awaited. Advised to call 999 if any noisy breathing or difficulty breathing.
```

Referral #019

```
REFERRING PRACTICE: PRAC-WLW-931
PRIORITY: URGENT
PATIENT: TEST-936783

60-year-old man with a two-week history of new daily headaches, worse on waking, with one episode of vomiting. No previous headache history. No head injury. Neurological exam: no focal deficit. Fundoscopy inconclusive (small pupils). BP 132/84. Advised to attend ED immediately if new weakness, confusion, or sudden severe headache.

CLINICIAN NOTE: Please consider same-week imaging.
```

Referral #020

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REFERRING PRACTICE: PRAC-HGT-837
PRIORITY: ROUTINE
PATIENT: TEST-188079

76-year-old man with a two-year history of worsening bilateral knee pain, now limiting walking to 200 metres. Osteoarthritis on X-ray 2024. Regular paracetamol and topical NSAID with limited effect. Weight-bearing X-ray shows moderate medial compartment narrowing bilaterally. Advised to return if knee becomes hot, swollen or unable to weight-bear.
```

Referral #021

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REFERRING PRACTICE: PRAC-WLW-603
PRIORITY: URGENT
PATIENT: TEST-200945

67-year-old man with a hard, painless left testicular swelling noticed three weeks
ago. No urinary symptoms. No previous scrotal problems. Examination: firm
irregular mass within the body of the testis, separate from epididymis. Patient
informed of suspected cancer referral.
```

Referral #022

```
REFERRING PRACTICE: PRAC-BRK-819
PRIORITY: ROUTINE

42-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Psoriasis since teens; new joint pain in fingers over last 6 months.
Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek
same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Query psoriatic arthritis; consider joint rheumatology/dermatology
clinic.
```

Referral #023

```
REFERRING PRACTICE: PRAC-CLV-331
PRIORITY: ROUTINE
PATIENT: TEST-832547

82-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor
stream for a year. Started tamsulosin six months ago with partial benefit. No
haematuria. PSA 2.1 ng/mL. DRE: smooth, moderately enlarged, benign-feeling
prostate. Renal function normal. Advised to attend ED if unable to pass urine.
```

Referral #024

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REFERRING PRACTICE: PRAC-BRK-767
PATIENT: TEST-536990

82-year-old woman with post-menopausal bleeding, two episodes over the past month.
On tamoxifen following breast cancer treatment in 2023. Speculum: cervix looks
normal. Smear up to date and normal. Told to expect contact within two weeks.
```

Referral #025

```
REFERRING PRACTICE: PRAC-RVS-843
PRIORITY: ROUTINE
PATIENT: TEST-232374

32-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor stream for a year. Started tamsulosin six months ago with partial benefit. No haematuria. PSA 2.1 ng/mL. DRE: smooth, moderately enlarged, benign-feeling prostate. Renal function normal. Advised to attend ED if unable to pass urine.
```

Referral #026

```
REFERRING PRACTICE: PRAC-WLW-516
PRIORITY: URGENT
PATIENT: TEST-401742

32-year-old man with dysphagia to solids, progressive over six weeks, food sticking at mid-chest. Barrett's oesophagus diagnosed 2018, lost to surveillance follow-up. FBC shows mild microcytic anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #027

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REFERRING PRACTICE: PRAC-OAK-851
PRIORITY: ROUTINE
PATIENT: TEST-430300

44-year-old man with a two-year history of difficulty concentrating, disorganisation and impulsivity, seeking ADHD assessment. Lost two jobs in the past year due to missed deadlines. ASRS screening questionnaire strongly positive. BP and pulse normal. No risk issues identified.
```

Referral #028

```
REFERRING PRACTICE: PRAC-RVS-884
PRIORITY: URGENT
PATIENT: TEST-531392

65-year-old woman with new-onset atrial fibrillation and symptomatic breathlessness on mild exertion. Hypertension, previous TIA in 2022. CHA2DS2-VASc 5. ECG confirms AF, rate 118. Started bisoprolol and apixaban. TFTs normal. Advised to call 999 for chest pain, collapse or worsening breathlessness.
```

Referral #029

```
REFERRING PRACTICE: PRAC-HGT-897

61-year-old man with a hard, painless left testicular swelling noticed three weeks ago. No trauma. Previous orchidopexy for undescended testis as a child. Examination: firm irregular mass within the body of the testis, separate from epididymis. Patient informed of suspected cancer referral.
```

Referral #030

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REFERRING PRACTICE: PRAC-STN-569
PRIORITY: ROUTINE
PATIENT: TEST-722773

34-year-old woman with heavy menstrual bleeding for over a year, flooding and changing pads hourly on heavy days. Mirena coil removed last year due to side effects. Hb 104, ferritin 9 - started on ferrous sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help if bleeding very heavy with dizziness.
```

Referral #031

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REFERRING PRACTICE: PRAC-MPL-660
PRIORITY: URGENT
PATIENT: TEST-484318

62-year-old woman with post-menopausal bleeding, two episodes over the past month. On tamoxifen following breast cancer treatment in 2023. Speculum: cervix looks normal. Smear up to date and normal. Told to expect contact within two weeks.

CLINICIAN NOTE: Patient requests a female clinician where possible.
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Referral #032

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REFERRING PRACTICE: PRAC-OAK-518
PRIORITY: URGENT
PATIENT: TEST-586504

Referred as discussed.
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Referral #033

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REFERRING PRACTICE: PRAC-HGT-991
PRIORITY: ROUTINE

48-year-old man with gallstones on ultrasound and three episodes of biliary colic in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #034

```
REFERRING PRACTICE: PRAC-WLW-278
PRIORITY: ROUTINE
PATIENT: TEST-773447

29-year-old man with eight months of fatigue and low mood not responding to
sertraline 100 mg. Previous trial of citalopram ineffective. Engaged with talking
therapies, completed course. Bloods including TFTs, B12 and ferritin normal. PHQ-9
18. Crisis line number given; advised to contact 111 option 2 or attend ED if
thoughts of self-harm.
```

Referral #035

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REFERRING PRACTICE: PRAC-CLV-324
PRIORITY: ROUTINE
PATIENT: TEST-943627

67-year-old man with a carpal tunnel picture: numbness of thumb, index and middle
fingers, waking at night. Hypothyroid on levothyroxine. Works on a production
line. Positive Phalen's and Tinel's. No thenar wasting. Steroid injection
declined. To return if weakness or wasting develops.
```

Referral #036

```
REFERRING PRACTICE: PRAC-RVS-349
PRIORITY: URGENT
PATIENT: TEST-374731

46-year-old man with a neck lump present for five weeks, slowly enlarging. Smoker;
recent dental treatment but lump predates this. 3 cm firm, non-tender level II
lymph node on the right. FBC and CRP normal. Patient aware of urgent suspected
cancer referral.
```

Referral #037

```
REFERRING PRACTICE: PRAC-OAK-966
PRIORITY: URGENT
PATIENT: TEST-530191

48-year-old man with three weeks of painless visible haematuria, two episodes.
Ex-smoker, 30 pack-years. Hypertension on amlodipine. Urine dip positive for
blood, MSU negative for infection. U&E normal, eGFR 74. Advised to attend ED if
unable to pass urine or passing clots.
```

Referral #038

```
REFERRING PRACTICE: PRAC-STN-706
PRIORITY: URGENT
PATIENT: TEST-585823

44-year-old woman with a swollen, hot, painful right calf for three days. Recent
knee arthroscopy two weeks ago. Wells score 3. D-dimer raised. Started on apixaban
treatment dose pending scan. Advised 999 for chest pain or breathlessness.
```

Referral #039

```
REFERRING PRACTICE: PRAC-WLW-192
PATIENT: TEST-594234

28-year-old woman with a new breast lump noticed four weeks ago, right upper outer
quadrant. No family history of breast or ovarian cancer. Post-menopausal, not on
HRT. Examination: 2 cm firm, non-tender, mobile lump. No axillary lymphadenopathy
palpable. Patient aware this is a suspected cancer referral and to expect contact
within two weeks.

CLINICIAN NOTE: Patient very anxious; please contact by phone rather than letter.
```

Referral #040

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REFERRING PRACTICE: PRAC-STN-548
PRIORITY: URGENT
PATIENT: TEST-637913

71-year-old woman with a swollen, hot, painful right calf for three days. Recent
knee arthroscopy two weeks ago. Wells score 3. D-dimer raised. Started on apixaban
treatment dose pending scan. Advised 999 for chest pain or breathlessness.
```

Referral #041

```
REFERRING PRACTICE: PRAC-HGT-831
PRIORITY: ROUTINE
PATIENT: TEST-884862

65-year-old woman with a two-year history of difficulty concentrating,
disorganisation and impulsivity, seeking ADHD assessment. School reports describe
similar difficulties. No substance misuse. ASRS screening questionnaire strongly
positive. BP and pulse normal. No risk issues identified.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #042

```
REFERRING PRACTICE: PRAC-RVS-759
PRIORITY: URGENT
PATIENT: TEST-348638

71-year-old woman with new-onset atrial fibrillation and symptomatic
breathlessness on mild exertion. No previous cardiac history. Drinks heavily at
weekends. ECG confirms AF, rate 118. Started bisoprolol and apixaban. TFTs normal.
Advised to call 999 for chest pain, collapse or worsening breathlessness.
```

Referral #043

```
REFERRING PRACTICE: PRAC-BRK-923
PRIORITY: URGENT
PATIENT: TEST-546208

50-year-old man with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #044

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REFERRING PRACTICE: PRAC-RVS-146
PRIORITY: URGENT
PATIENT: TEST-248038

Thank you for seeing this patient.
```

Referral #045

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REFERRING PRACTICE: PRAC-OAK-651

80-year-old woman with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #046

```
REFERRING PRACTICE: PRAC-STN-371
PRIORITY: ROUTINE
PATIENT: TEST-334138

29-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Psoriasis since teens; new joint pain in fingers over last 6 months.
Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek
same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #047

```
REFERRING PRACTICE: PRAC-RVS-664
PRIORITY: ROUTINE
PATIENT: TEST-214569

30-year-old man with a two-year history of difficulty concentrating,
disorganisation and impulsivity, seeking ADHD assessment. Lost two jobs in the
past year due to missed deadlines. ASRS screening questionnaire strongly positive.
BP and pulse normal. No risk issues identified.
```

Referral #048

```
REFERRING PRACTICE: PRAC-BRK-476
PRIORITY: ROUTINE

55-year-old woman with lower back pain radiating to the left leg for 14 weeks.
Previous episode 5 years ago settled with physio. Straight leg raise positive at
40 degrees. Power and reflexes normal. No red flags. Cauda equina safety-netting
given verbally and in writing.
```

Referral #049

```
REFERRING PRACTICE: PRAC-BRK-864
PRIORITY: ROUTINE
PATIENT: TEST-716215

43-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. BMI 36. HGV driver - advised about DVLA
obligations. STOP-BANG 6. Thyroid function normal. Advised not to drive if feeling
sleepy.

CLINICIAN NOTE: Occupational driver; would appreciate expedited assessment.
```

Referral #050

```
REFERRING PRACTICE: PRAC-WLW-392
PRIORITY: ROUTINE
PATIENT: TEST-335760

40-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Hypothyroid on levothyroxine. Works on a
production line. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #051

```
REFERRING PRACTICE: PRAC-HGT-235
PATIENT: TEST-732608

61-year-old woman with sudden painless loss of vision in the right eye this
morning, now partially recovered. No previous eye problems. Takes no regular
medication. Visual acuity 6/12 right, 6/6 left. ECG sinus rhythm. Started aspirin
300 mg. Advised 999 if any weakness, speech disturbance or further visual loss.

CLINICIAN NOTE: Possible amaurosis fugax - TIA pathway may be more appropriate if
not accepted.
```

Referral #052

```
REFERRING PRACTICE: PRAC-OAK-656
PRIORITY: URGENT
PATIENT: TEST-368159

57-year-old man with a pigmented lesion on the back that has changed shape and
bled over the last two months. Previous BCC excised from nose in 2021. Dermoscopy:
asymmetric, irregular border, multiple colours, 9 mm. 7-point checklist score 5.
Patient informed of suspected cancer pathway.

CLINICIAN NOTE: Photographs attached to the e-RS referral.
```

Referral #053

```
REFERRING PRACTICE: PRAC-CLV-686
PRIORITY: ROUTINE
PATIENT: TEST-296022

Kindly review.
```

Referral #054

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REFERRING PRACTICE: PRAC-CLV-698
PRIORITY: ROUTINE
PATIENT: TEST-481977

Please see.
```

Referral #055

```
REFERRING PRACTICE: PRAC-WLW-783
PRIORITY: ROUTINE

68-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #056

```
REFERRING PRACTICE: PRAC-HGT-666
PRIORITY: ROUTINE
PATIENT: TEST-225761

83-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #057

```
REFERRING PRACTICE: PRAC-MPL-912
PRIORITY: URGENT
PATIENT: TEST-304176

44-year-old woman with a two-week history of new daily headaches, worse on waking,
with one episode of vomiting. Known migraine but describes these as different in
character. Neurological exam: no focal deficit. Fundoscopy inconclusive (small
pupils). BP 132/84. Advised to attend ED immediately if new weakness, confusion,
or sudden severe headache.

CLINICIAN NOTE: Please consider same-week imaging.
```

Referral #058

```
REFERRING PRACTICE: PRAC-STN-363
PRIORITY: ROUTINE
PATIENT: TEST-901609

82-year-old woman with gradually worsening hearing in the left ear over a year,
with intermittent tinnitus. No vertigo or ear discharge. No previous ear surgery.
Otoscopy normal both sides. Rinne positive bilaterally, Weber lateralises to the
right. To return urgently if sudden further loss of hearing or facial weakness.

CLINICIAN NOTE: Asymmetric loss; please consider MRI IAMs.
```

Referral #059

```
REFERRING PRACTICE: PRAC-STN-643
PRIORITY: URGENT
PATIENT: TEST-110832

81-year-old man with three weeks of painless visible haematuria, two episodes. Ex-smoker, 30 pack-years. Hypertension on amlodipine. Urine dip positive for blood, MSU negative for infection. U&E normal, eGFR 74. Advised to attend ED if unable to pass urine or passing clots.
```

Referral #060

```
REFERRING PRACTICE: PRAC-STN-677
PRIORITY: URGENT
PATIENT: TEST-881589

75-year-old woman with dysphagia to solids, progressive over six weeks, food sticking at mid-chest. Barrett's oesophagus diagnosed 2018, lost to surveillance follow-up. FBC shows mild microcytic anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #061

```
REFERRING PRACTICE: PRAC-OAK-519
PRIORITY: ROUTINE
PATIENT: TEST-682754

82-year-old woman with recurrent heartburn and regurgitation despite eight weeks of full-dose omeprazole. Hiatus hernia suspected on previous barium swallow (2019). FBC normal. Lifestyle advice given; trial of alginate added. Told to seek review promptly if difficulty swallowing, vomiting blood or black stools.
```

Referral #062

```
REFERRING PRACTICE: PRAC-OAK-366
PRIORITY: ROUTINE
PATIENT: TEST-193841

77-year-old woman with heavy menstrual bleeding for over a year, flooding and changing pads hourly on heavy days. Mirena coil removed last year due to side effects. Hb 104, ferritin 9 - started on ferrous sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help if bleeding very heavy with dizziness.
```

Referral #063

```
REFERRING PRACTICE: PRAC-STN-982
PRIORITY: URGENT
PATIENT: TEST-251711

84-year-old woman with persistent cough for six weeks and two episodes of
haemoptysis. Smoker, 40 pack-years. COPD. Chest X-ray: 3 cm right upper lobe
opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #064

```
REFERRING PRACTICE: PRAC-RVS-360
PRIORITY: ROUTINE
PATIENT: TEST-950390

31-year-old man with eight months of fatigue and low mood not responding to
sertraline 100 mg. Second episode of depression. No current suicidal ideation;
protective factors include children. Bloods including TFTs, B12 and ferritin
normal. PHQ-9 18. Crisis line number given; advised to contact 111 option 2 or
attend ED if thoughts of self-harm.
```

Referral #065

```
REFERRING PRACTICE: PRAC-CLV-484
PRIORITY: ROUTINE
PATIENT: TEST-633001

53-year-old woman with gallstones on ultrasound and three episodes of biliary
colic in four months. One A&E attendance last month; discharged with analgesia.
LFTs normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised
to attend ED if fever, jaundice or pain lasting over 6 hours.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #066

```
REFERRING PRACTICE: PRAC-STN-511
PRIORITY: ROUTINE
PATIENT: TEST-599024

51-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Symptoms for 9 months. Night splints for 3 months
gave partial relief. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #067

```
REFERRING PRACTICE: PRAC-MPL-456
PATIENT: TEST-722157

51-year-old woman with recurrent heartburn and regurgitation despite eight weeks
of full-dose omeprazole. No weight loss, dysphagia or anaemia. H. pylori stool
antigen negative. FBC normal. Lifestyle advice given; trial of alginate added.
Told to seek review promptly if difficulty swallowing, vomiting blood or black
stools.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #068

```
REFERRING PRACTICE: PRAC-CLV-714
PRIORITY: ROUTINE
PATIENT: TEST-605843

Kindly review.
```

Referral #069

```
REFERRING PRACTICE: PRAC-STN-608
PRIORITY: ROUTINE
PATIENT: TEST-906533

79-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Adherent to ICS/LABA (inhaler technique checked). Eosinophils 0.6.
Peak flow 68% of predicted. Spirometry shows reversible obstruction. Personal
asthma action plan updated; advised 999 if not improving with reliever.
```

Referral #070

```
REFERRING PRACTICE: PRAC-BRK-821
PRIORITY: ROUTINE

79-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Symptoms for 9 months. Night splints for 3 months
gave partial relief. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #071

```
REFERRING PRACTICE: PRAC-MPL-427
PRIORITY: ROUTINE
PATIENT: TEST-298239

69-year-old woman with lower back pain radiating to the left leg for 14 weeks.
Office worker. Completed physiotherapy; still struggling with sitting. Straight
leg raise positive at 40 degrees. Power and reflexes normal. No red flags. Cauda
equina safety-netting given verbally and in writing.
```

Referral #072

```
REFERRING PRACTICE: PRAC-RVS-611
PRIORITY: URGENT
PATIENT: TEST-205278

43-year-old man with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #073

```
REFERRING PRACTICE: PRAC-OAK-207
PRIORITY: ROUTINE
PATIENT: TEST-555914

75-year-old woman with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Missing significant time from work. No quinsy. Currently
well between episodes; tonsils grade 2. Advised to seek urgent review if unable to
swallow or one-sided throat swelling.
```

Referral #074

```
REFERRING PRACTICE: PRAC-BRK-397
PRIORITY: ROUTINE
PATIENT: TEST-469226

32-year-old woman with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Missing significant time from work. No quinsy. Currently
well between episodes; tonsils grade 2. Advised to seek urgent review if unable to
swallow or one-sided throat swelling.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #075

```
REFERRING PRACTICE: PRAC-STN-258
PRIORITY: URGENT
PATIENT: TEST-718851

39-year-old man with a two-week history of new daily headaches, worse on waking,
with one episode of vomiting. Known migraine but describes these as different in
character. Neurological exam: no focal deficit. Fundoscopy inconclusive (small
pupils). BP 132/84. Advised to attend ED immediately if new weakness, confusion,
or sudden severe headache.

CLINICIAN NOTE: Please consider same-week imaging.
```

Referral #076

```
REFERRING PRACTICE: PRAC-WLW-800
PRIORITY: URGENT
PATIENT: TEST-191997

47-year-old man with a hard, painless left testicular swelling noticed three weeks
ago. No urinary symptoms. No previous scrotal problems. Examination: firm
irregular mass within the body of the testis, separate from epididymis. Patient
informed of suspected cancer referral.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #077

```
REFERRING PRACTICE: PRAC-WLW-437
PRIORITY: URGENT
PATIENT: TEST-945727

79-year-old man with persistent cough for six weeks and two episodes of
haemoptysis. Smoker, 40 pack-years. COPD. Chest X-ray: 3 cm right upper lobe
opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #078

```
REFERRING PRACTICE: PRAC-OAK-873
PRIORITY: ROUTINE
PATIENT: TEST-811463

Kindly review.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #079

```
REFERRING PRACTICE: PRAC-WLW-993
PRIORITY: ROUTINE
PATIENT: TEST-358267

37-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Childhood asthma, worsened after moving to a damp flat. Peak flow
68% of predicted. Spirometry shows reversible obstruction. Personal asthma action
plan updated; advised 999 if not improving with reliever.
```

Referral #080

```
REFERRING PRACTICE: PRAC-MPL-434
PRIORITY: URGENT
PATIENT: TEST-372180

38-year-old man with a hard, painless left testicular swelling noticed three weeks
ago. No urinary symptoms. No previous scrotal problems. Examination: firm
irregular mass within the body of the testis, separate from epididymis. Patient
informed of suspected cancer referral.
```

Referral #081

```
REFERRING PRACTICE: PRAC-HGT-433
PRIORITY: ROUTINE
PATIENT: TEST-551348

54-year-old woman with a two-year history of difficulty concentrating,
disorganisation and impulsivity, seeking ADHD assessment. Lost two jobs in the
past year due to missed deadlines. ASRS screening questionnaire strongly positive.
BP and pulse normal. No risk issues identified.
```

Referral #082

```
REFERRING PRACTICE: PRAC-BRK-221

72-year-old woman with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #083

```
REFERRING PRACTICE: PRAC-OAK-656
PRIORITY: URGENT
PATIENT: TEST-554290

69-year-old woman with a swollen, hot, painful right calf for three days. Recent
knee arthroscopy two weeks ago. Wells score 3. D-dimer raised. Started on apixaban
treatment dose pending scan. Advised 999 for chest pain or breathlessness.
```

Referral #084

```
REFERRING PRACTICE: PRAC-HGT-140
PRIORITY: URGENT
PATIENT: TEST-277372

38-year-old woman with a pigmented lesion on the back that has changed shape and
bled over the last two months. Fair skin, multiple sunburns as a child. No
personal history of skin cancer. Dermoscopy: asymmetric, irregular border,
multiple colours, 9 mm. 7-point checklist score 5. Patient informed of suspected
cancer pathway.
```

Referral #085

```
REFERRING PRACTICE: PRAC-CLV-569
PRIORITY: ROUTINE
PATIENT: TEST-233209

61-year-old man with chronic plaque psoriasis covering roughly 15% of body surface
area. Psoriasis since teens; new joint pain in fingers over last 6 months. Bloods:
LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek same-day
review if skin becomes widespread red and hot.
```

Referral #086

```
REFERRING PRACTICE: PRAC-WLW-402
PRIORITY: ROUTINE
PATIENT: TEST-709193

71-year-old woman with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #087

```
REFERRING PRACTICE: PRAC-HGT-834
PRIORITY: URGENT
PATIENT: TEST-644362

72-year-old man with a swollen, hot, painful right calf for three days. Recent
knee arthroscopy two weeks ago. Wells score 3. D-dimer raised. Started on apixaban
treatment dose pending scan. Advised 999 for chest pain or breathlessness.
```

Referral #088

```
REFERRING PRACTICE: PRAC-RVS-529
PRIORITY: ROUTINE
PATIENT: TEST-256950

68-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Mirena coil removed last year due to side
effects. Hb 104, ferritin 9 - started on ferrous sulphate. Pelvic ultrasound: 3 cm
intramural fibroid. Advised to seek urgent help if bleeding very heavy with
dizziness.
```

Referral #089

```
REFERRING PRACTICE: PRAC-MPL-692
PRIORITY: ROUTINE
PATIENT: TEST-947597

68-year-old woman with a two-year history of worsening bilateral knee pain, now
limiting walking to 200 metres. Osteoarthritis on X-ray 2024. Regular paracetamol
and topical NSAID with limited effect. Weight-bearing X-ray shows moderate medial
compartment narrowing bilaterally. Advised to return if knee becomes hot, swollen
or unable to weight-bear.
```

Referral #090

```
REFERRING PRACTICE: PRAC-STN-193
PRIORITY: ROUTINE
PATIENT: TEST-104181

79-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Childhood asthma, worsened after moving to a damp flat. Peak flow
68% of predicted. Spirometry shows reversible obstruction. Personal asthma action
plan updated; advised 999 if not improving with reliever.
```

Referral #091

```
REFERRING PRACTICE: PRAC-BRK-864
PRIORITY: URGENT
PATIENT: TEST-430217

84-year-old man with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Barrett's oesophagus diagnosed 2018, lost to surveillance
follow-up. FBC shows mild microcytic anaemia, Hb 112. Advised to attend ED if
unable to swallow fluids or saliva.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #092

```
REFERRING PRACTICE: PRAC-HGT-457
PRIORITY: ROUTINE
PATIENT: TEST-726433

50-year-old man with eight months of fatigue and low mood not responding to
sertraline 100 mg. Second episode of depression. No current suicidal ideation;
protective factors include children. Bloods including TFTs, B12 and ferritin
normal. PHQ-9 18. Crisis line number given; advised to contact 111 option 2 or
attend ED if thoughts of self-harm.
```

Referral #093

```
REFERRING PRACTICE: PRAC-BRK-752
PRIORITY: ROUTINE
PATIENT: TEST-773527

47-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor
stream for a year. Started tamsulosin six months ago with partial benefit. No
haematuria. PSA 2.1 ng/mL. DRE: smooth, moderately enlarged, benign-feeling
prostate. Renal function normal. Advised to attend ED if unable to pass urine.
```

Referral #094

```
REFERRING PRACTICE: PRAC-WLW-240

57-year-old man with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #095

```
REFERRING PRACTICE: PRAC-CLV-269
PRIORITY: ROUTINE
PATIENT: TEST-101997

48-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #096

```
REFERRING PRACTICE: PRAC-STN-382
PRIORITY: URGENT
PATIENT: TEST-272674

52-year-old woman with unintentional weight loss of 8 kg over three months and new
change in bowel habit to looser stools. Father had colorectal cancer in his 60s.
Otherwise fit and well. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin 6. Abdominal
exam unremarkable. Advised to attend ED with any rectal bleeding causing dizziness
or severe abdominal pain.

CLINICIAN NOTE: FIT positive above threshold.
CLINICIAN NOTE: Lives alone; daughter is NOK and happy to be contacted.
```

Referral #097

```
REFERRING PRACTICE: PRAC-CLV-927
PRIORITY: ROUTINE
PATIENT: TEST-949807

32-year-old woman with recurrent heartburn and regurgitation despite eight weeks
of full-dose omeprazole. No weight loss, dysphagia or anaemia. H. pylori stool
antigen negative. FBC normal. Lifestyle advice given; trial of alginate added.
Told to seek review promptly if difficulty swallowing, vomiting blood or black
stools.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #098

```
REFERRING PRACTICE: PRAC-OAK-268
PRIORITY: URGENT
PATIENT: TEST-976360

43-year-old woman with a new breast lump noticed four weeks ago, right upper outer
quadrant. No family history of breast or ovarian cancer. Post-menopausal, not on
HRT. Examination: 2 cm firm, non-tender, mobile lump. No axillary lymphadenopathy
palpable. Patient aware this is a suspected cancer referral and to expect contact
within two weeks.

CLINICIAN NOTE: Patient very anxious; please contact by phone rather than letter.
```

Referral #099

```
REFERRING PRACTICE: PRAC-STN-389
PRIORITY: URGENT
PATIENT: TEST-955749

61-year-old man with persistent cough for six weeks and two episodes of
haemoptysis. Smoker, 40 pack-years. COPD. Chest X-ray: 3 cm right upper lobe
opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #100

```
REFERRING PRACTICE: PRAC-MPL-598
PRIORITY: URGENT
PATIENT: TEST-776849

82-year-old woman with persistent cough for six weeks and two episodes of
haemoptysis. Smoker, 40 pack-years. COPD. Chest X-ray: 3 cm right upper lobe
opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #101

```
REFERRING PRACTICE: PRAC-BRK-432
PRIORITY: ROUTINE

50-year-old woman with eight months of fatigue and low mood not responding to
sertraline 100 mg. Previous trial of citalopram ineffective. Engaged with talking
therapies, completed course. Bloods including TFTs, B12 and ferritin normal. PHQ-9
18. Crisis line number given; advised to contact 111 option 2 or attend ED if
thoughts of self-harm.
```

Referral #102

```
REFERRING PRACTICE: PRAC-MPL-926
PRIORITY: URGENT
PATIENT: TEST-741107

68-year-old man with a swollen, hot, painful right calf for three days. Recent
knee arthroscopy two weeks ago. Wells score 3. D-dimer raised. Started on apixaban
treatment dose pending scan. Advised 999 for chest pain or breathlessness.
```

Referral #103

```
REFERRING PRACTICE: PRAC-CLV-373
PRIORITY: URGENT
PATIENT: TEST-515479

49-year-old woman with post-menopausal bleeding, two episodes over the past month.
On tamoxifen following breast cancer treatment in 2023. Speculum: cervix looks
normal. Smear up to date and normal. Told to expect contact within two weeks.
```

Referral #104

```
REFERRING PRACTICE: PRAC-CLV-963
PRIORITY: ROUTINE
PATIENT: TEST-824230

66-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #105

```
REFERRING PRACTICE: PRAC-STN-444
PRIORITY: URGENT
PATIENT: TEST-649086

78-year-old woman with a neck lump present for five weeks, slowly enlarging.
Smoker; recent dental treatment but lump predates this. 3 cm firm, non-tender
level II lymph node on the right. FBC and CRP normal. Patient aware of urgent
suspected cancer referral.
```

Referral #106

```
REFERRING PRACTICE: PRAC-CLV-508
PRIORITY: ROUTINE
PATIENT: TEST-137446

54-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. Epworth score 15. Collar size 18 inches.
Hypertension. STOP-BANG 6. Thyroid function normal. Advised not to drive if
feeling sleepy.
```

Referral #107

```
REFERRING PRACTICE: PRAC-STN-511
PRIORITY: URGENT
PATIENT: TEST-893375

73-year-old man with a two-week history of new daily headaches, worse on waking,
with one episode of vomiting. No previous headache history. No head injury.
Neurological exam: no focal deficit. Fundoscopy inconclusive (small pupils). BP
132/84. Advised to attend ED immediately if new weakness, confusion, or sudden
severe headache.

CLINICIAN NOTE: Please consider same-week imaging.
```

Referral #108

```
REFERRING PRACTICE: PRAC-CLV-162
PRIORITY: URGENT

33-year-old woman with unintentional weight loss of 8 kg over three months and new change in bowel habit to looser stools. Father had colorectal cancer in his 60s. Otherwise fit and well. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin 6. Abdominal exam unremarkable. Advised to attend ED with any rectal bleeding causing dizziness or severe abdominal pain.

CLINICIAN NOTE: Lives alone; daughter is NOK and happy to be contacted.
```

Referral #109

```
REFERRING PRACTICE: PRAC-RVS-734
PRIORITY: URGENT
PATIENT: TEST-357876

53-year-old man with persistent cough for six weeks and two episodes of haemoptysis. Asbestos exposure as a shipyard worker in the 1980s. Chest X-ray: 3 cm right upper lobe opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #110

```
REFERRING PRACTICE: PRAC-MPL-717
PRIORITY: URGENT
PATIENT: TEST-117453

52-year-old man with a neck lump present for five weeks, slowly enlarging. No recent infections. Non-smoker. Night sweats for two weeks. 3 cm firm, non-tender level II lymph node on the right. FBC and CRP normal. Patient aware of urgent suspected cancer referral.
```

Referral #111

```
REFERRING PRACTICE: PRAC-HGT-123

31-year-old man with recurrent tonsillitis: seven episodes in the past year requiring antibiotics. Episodes documented in records for the past two years (5 and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek urgent review if unable to swallow or one-sided throat swelling.
```

Referral #112

```
REFERRING PRACTICE: PRAC-HGT-512
PRIORITY: ROUTINE
PATIENT: TEST-974099

47-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Failed potent topical steroids and vitamin D analogue over 12
months. DLQI 16. Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative.
Advised to seek same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Query psoriatic arthritis; consider joint rheumatology/dermatology
clinic.
```

Referral #113

```
REFERRING PRACTICE: PRAC-WLW-912
PRIORITY: URGENT
PATIENT: TEST-658483

28-year-old woman with a swollen, hot, painful right calf for three days.
Long-haul flight last week. On combined pill. Wells score 3. D-dimer raised.
Started on apixaban treatment dose pending scan. Advised 999 for chest pain or
breathlessness.
```

Referral #114

```
REFERRING PRACTICE: PRAC-CLV-607
PRIORITY: ROUTINE
PATIENT: TEST-554687

69-year-old man with poorly controlled asthma: two courses of oral steroids in the
last year. Childhood asthma, worsened after moving to a damp flat. Peak flow 68%
of predicted. Spirometry shows reversible obstruction. Personal asthma action plan
updated; advised 999 if not improving with reliever.
```

Referral #115

```
REFERRING PRACTICE: PRAC-HGT-454
PRIORITY: ROUTINE
PATIENT: TEST-273873

72-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor
stream for a year. Started tamsulosin six months ago with partial benefit. No
haematuria. PSA 2.1 ng/mL. DRE: smooth, moderately enlarged, benign-feeling
prostate. Renal function normal. Advised to attend ED if unable to pass urine.
```

Referral #116

```
REFERRING PRACTICE: PRAC-RVS-235
PRIORITY: ROUTINE
PATIENT: TEST-621157

Referred as discussed.
```

Referral #117

```
REFERRING PRACTICE: PRAC-OAK-133

74-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Hypothyroid on levothyroxine. Works on a
production line. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #118

```
REFERRING PRACTICE: PRAC-BRK-973
PRIORITY: URGENT
PATIENT: TEST-259753

As per phone call.
```

Referral #119

```
REFERRING PRACTICE: PRAC-HGT-488
PRIORITY: ROUTINE
PATIENT: TEST-209853

46-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Mirena coil removed last year due to side
effects. Hb 104, ferritin 9 - started on ferrous sulphate. Pelvic ultrasound: 3 cm
intramural fibroid. Advised to seek urgent help if bleeding very heavy with
dizziness.
```

Referral #120

```
REFERRING PRACTICE: PRAC-CLV-620
PRIORITY: ROUTINE
PATIENT: TEST-213442

81-year-old woman with lower back pain radiating to the left leg for 14 weeks.
Previous episode 5 years ago settled with physio. Straight leg raise positive at
40 degrees. Power and reflexes normal. No red flags. Cauda equina safety-netting
given verbally and in writing.
```

Referral #121

```
REFERRING PRACTICE: PRAC-BRK-876
PRIORITY: ROUTINE
PATIENT: TEST-167243

75-year-old man with poorly controlled asthma: two courses of oral steroids in the
last year. Childhood asthma, worsened after moving to a damp flat. Peak flow 68%
of predicted. Spirometry shows reversible obstruction. Personal asthma action plan
updated; advised 999 if not improving with reliever.
```

Referral #122

```
REFERRING PRACTICE: PRAC-STN-572
PRIORITY: ROUTINE
PATIENT: TEST-264998

79-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor
stream for a year. IPSS 19. No family history of prostate cancer. PSA 2.1 ng/mL.
DRE: smooth, moderately enlarged, benign-feeling prostate. Renal function normal.
Advised to attend ED if unable to pass urine.
```

Referral #123

```
REFERRING PRACTICE: PRAC-STN-384

39-year-old man with sudden painless loss of vision in the right eye this morning,
now partially recovered. No previous eye problems. Takes no regular medication.
Visual acuity 6/12 right, 6/6 left. ECG sinus rhythm. Started aspirin 300 mg.
Advised 999 if any weakness, speech disturbance or further visual loss.
```

Referral #124

```
REFERRING PRACTICE: PRAC-STN-181
PRIORITY: URGENT
PATIENT: TEST-513274

61-year-old woman with a new breast lump noticed four weeks ago, right upper outer
quadrant. Mother diagnosed with breast cancer aged 52. On combined oral
contraceptive. Examination: 2 cm firm, non-tender, mobile lump. No axillary
lymphadenopathy palpable. Patient aware this is a suspected cancer referral and to
expect contact within two weeks.

CLINICIAN NOTE: Patient very anxious; please contact by phone rather than letter.
```

Referral #125

```
REFERRING PRACTICE: PRAC-CLV-999
PRIORITY: ROUTINE
PATIENT: TEST-275083

76-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. Epworth score 15. Collar size 18 inches.
Hypertension. STOP-BANG 6. Thyroid function normal. Advised not to drive if
feeling sleepy.

CLINICIAN NOTE: Occupational driver; would appreciate expedited assessment.
```

Referral #126

```
REFERRING PRACTICE: PRAC-STN-347
PRIORITY: ROUTINE
PATIENT: TEST-732834

82-year-old woman with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #127

```
REFERRING PRACTICE: PRAC-WLW-243
PRIORITY: ROUTINE
PATIENT: TEST-551601

Thank you for seeing this patient.
```

Referral #128

```
REFERRING PRACTICE: PRAC-OAK-409
PRIORITY: URGENT
PATIENT: TEST-305873

82-year-old woman with new-onset atrial fibrillation and symptomatic
breathlessness on mild exertion. Hypertension, previous TIA in 2022. CHA2DS2-VASc
5. ECG confirms AF, rate 118. Started bisoprolol and apixaban. TFTs normal.
Advised to call 999 for chest pain, collapse or worsening breathlessness.
```

Referral #129

```
REFERRING PRACTICE: PRAC-WLW-601
PRIORITY: ROUTINE
PATIENT: TEST-777649

61-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Failed potent topical steroids and vitamin D analogue over 12
months. DLQI 16. Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative.
Advised to seek same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Query psoriatic arthritis; consider joint rheumatology/dermatology
clinic.
```

Referral #130

```
REFERRING PRACTICE: PRAC-BRK-479
PRIORITY: ROUTINE
PATIENT: TEST-264084

56-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #131

```
REFERRING PRACTICE: PRAC-WLW-774

52-year-old woman with a carpal tunnel picture: numbness of thumb, index and
middle fingers, waking at night. Hypothyroid on levothyroxine. Works on a
production line. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #132

```
REFERRING PRACTICE: PRAC-WLW-426
PRIORITY: URGENT
PATIENT: TEST-752627

37-year-old woman with persistent cough for six weeks and two episodes of
haemoptysis. Smoker, 40 pack-years. COPD. Chest X-ray: 3 cm right upper lobe
opacity. Bloods normal. Advised to attend ED if coughing large volumes of blood.
```

Referral #133

```
REFERRING PRACTICE: PRAC-HGT-831
PRIORITY: URGENT
PATIENT: TEST-548609

As per phone call.
```

Referral #134

```
REFERRING PRACTICE: PRAC-OAK-746
PRIORITY: ROUTINE
PATIENT: TEST-171693

75-year-old man with a carpal tunnel picture: numbness of thumb, index and middle
fingers, waking at night. Symptoms for 9 months. Night splints for 3 months gave
partial relief. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #135

```
REFERRING PRACTICE: PRAC-OAK-172

42-year-old man with a persistent hoarse voice for four weeks. Current smoker, 20
per day. Drinks around 30 units a week. No neck lumps palpable. Chest X-ray
requested, result awaited. Advised to call 999 if any noisy breathing or
difficulty breathing.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #136

```
REFERRING PRACTICE: PRAC-BRK-128
PRIORITY: URGENT

31-year-old man with a hard, painless left testicular swelling noticed three weeks
ago. No trauma. Previous orchidopexy for undescended testis as a child.
Examination: firm irregular mass within the body of the testis, separate from
epididymis. Patient informed of suspected cancer referral.
```

Referral #137

```
REFERRING PRACTICE: PRAC-MPL-121
PRIORITY: ROUTINE
PATIENT: TEST-445991

57-year-old woman with gallstones on ultrasound and three episodes of biliary
colic in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #138

```
REFERRING PRACTICE: PRAC-BRK-562
PATIENT: TEST-333275

73-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #139

```
REFERRING PRACTICE: PRAC-HGT-791
PRIORITY: ROUTINE
PATIENT: TEST-504845

71-year-old woman with gallstones on ultrasound and three episodes of biliary
colic in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #140

```
REFERRING PRACTICE: PRAC-RVS-890
PRIORITY: URGENT
PATIENT: TEST-767008

71-year-old man with unintentional weight loss of 8 kg over three months and new
change in bowel habit to looser stools. Father had colorectal cancer in his 60s.
Otherwise fit and well. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin 6. Abdominal
exam unremarkable. Advised to attend ED with any rectal bleeding causing dizziness
or severe abdominal pain.
```

Referral #141

```
REFERRING PRACTICE: PRAC-HGT-327
PRIORITY: URGENT
PATIENT: TEST-767626

As per phone call.
```

Referral #142

```
REFERRING PRACTICE: PRAC-HGT-956
PRIORITY: URGENT
PATIENT: TEST-424874

51-year-old woman with a neck lump present for five weeks, slowly enlarging. No
recent infections. Non-smoker. Night sweats for two weeks. 3 cm firm, non-tender
level II lymph node on the right. FBC and CRP normal. Patient aware of urgent
suspected cancer referral.
```

Referral #143

```
REFERRING PRACTICE: PRAC-CLV-945
PRIORITY: ROUTINE
PATIENT: TEST-893118

34-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. Epworth score 15. Collar size 18 inches.
Hypertension. STOP-BANG 6. Thyroid function normal. Advised not to drive if
feeling sleepy.
```

Referral #144

```
REFERRING PRACTICE: PRAC-OAK-936
PRIORITY: URGENT
PATIENT: TEST-258606

29-year-old woman with new-onset atrial fibrillation and symptomatic
breathlessness on mild exertion. No previous cardiac history. Drinks heavily at
weekends. ECG confirms AF, rate 118. Started bisoprolol and apixaban. TFTs normal.
Advised to call 999 for chest pain, collapse or worsening breathlessness.
```

Referral #145

```
REFERRING PRACTICE: PRAC-STN-716
PRIORITY: URGENT
PATIENT: TEST-454681

29-year-old woman with three weeks of painless visible haematuria, two episodes.
Previous smoker, quit 2015. No prior urological history. Urine dip positive for
blood, MSU negative for infection. U&E normal, eGFR 74. Advised to attend ED if
unable to pass urine or passing clots.

CLINICIAN NOTE: Meets 2-week-wait criteria for suspected bladder cancer.
```

Referral #146

```
REFERRING PRACTICE: PRAC-WLW-939
PRIORITY: ROUTINE
PATIENT: TEST-498917

Kindly review.
```

Referral #147

```
REFERRING PRACTICE: PRAC-CLV-844
PRIORITY: URGENT
PATIENT: TEST-743069

70-year-old man with persistent cough for six weeks and two episodes of
haemoptysis. Asbestos exposure as a shipyard worker in the 1980s. Chest X-ray: 3
cm right upper lobe opacity. Bloods normal. Advised to attend ED if coughing large
volumes of blood.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #148

```
REFERRING PRACTICE: PRAC-BRK-310
PRIORITY: ROUTINE
PATIENT: TEST-228933

41-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Childhood asthma, worsened after moving to a damp flat. Peak flow
68% of predicted. Spirometry shows reversible obstruction. Personal asthma action
plan updated; advised 999 if not improving with reliever.
```

Referral #149

```
REFERRING PRACTICE: PRAC-RVS-139
PRIORITY: ROUTINE
PATIENT: TEST-598235

54-year-old woman with a two-year history of worsening bilateral knee pain, now
limiting walking to 200 metres. Osteoarthritis on X-ray 2024. Regular paracetamol
and topical NSAID with limited effect. Weight-bearing X-ray shows moderate medial
compartment narrowing bilaterally. Advised to return if knee becomes hot, swollen
or unable to weight-bear.
```

Referral #150

```
REFERRING PRACTICE: PRAC-CLV-646
PRIORITY: URGENT
PATIENT: TEST-366894

74-year-old woman with unintentional weight loss of 8 kg over three months and new
change in bowel habit to looser stools. Iron-deficiency anaemia picked up on
routine bloods; no prior GI history. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin
6. Abdominal exam unremarkable. Advised to attend ED with any rectal bleeding
causing dizziness or severe abdominal pain.

CLINICIAN NOTE: Lives alone; daughter is NOK and happy to be contacted.
CLINICIAN NOTE: FIT positive above threshold.
```

Referral #151

```
REFERRING PRACTICE: PRAC-RVS-764
PRIORITY: URGENT
PATIENT: TEST-442239

34-year-old woman with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #152

```
REFERRING PRACTICE: PRAC-CLV-762
PRIORITY: URGENT

76-year-old man with unintentional weight loss of 8 kg over three months and new
change in bowel habit to looser stools. Iron-deficiency anaemia picked up on
routine bloods; no prior GI history. FIT result 142 ug Hb/g. Hb 98 g/L, ferritin
6. Abdominal exam unremarkable. Advised to attend ED with any rectal bleeding
causing dizziness or severe abdominal pain.
```

Referral #153

```
REFERRING PRACTICE: PRAC-OAK-770
PRIORITY: ROUTINE

47-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Adherent to ICS/LABA (inhaler technique checked). Eosinophils 0.6.
Peak flow 68% of predicted. Spirometry shows reversible obstruction. Personal
asthma action plan updated; advised 999 if not improving with reliever.
```

Referral #154

```
REFERRING PRACTICE: PRAC-OAK-271
PRIORITY: ROUTINE

63-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. Epworth score 15. Collar size 18 inches.
Hypertension. STOP-BANG 6. Thyroid function normal. Advised not to drive if
feeling sleepy.

CLINICIAN NOTE: Occupational driver; would appreciate expedited assessment.
```

Referral #155

```
REFERRING PRACTICE: PRAC-HGT-730
PATIENT: TEST-413509

56-year-old man with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. Epworth score 15. Collar size 18 inches.
Hypertension. STOP-BANG 6. Thyroid function normal. Advised not to drive if
feeling sleepy.

CLINICIAN NOTE: Occupational driver; would appreciate expedited assessment.
```

Referral #156

```
REFERRING PRACTICE: PRAC-HGT-316
PRIORITY: URGENT
PATIENT: TEST-756781

81-year-old woman with persistent cough for six weeks and two episodes of
haemoptysis. Asbestos exposure as a shipyard worker in the 1980s. Chest X-ray: 3
cm right upper lobe opacity. Bloods normal. Advised to attend ED if coughing large
volumes of blood.
```

Referral #157

```
REFERRING PRACTICE: PRAC-WLW-655
PATIENT: TEST-666893

28-year-old woman with a neck lump present for five weeks, slowly enlarging. No
recent infections. Non-smoker. Night sweats for two weeks. 3 cm firm, non-tender
level II lymph node on the right. FBC and CRP normal. Patient aware of urgent
suspected cancer referral.
```

Referral #158

```
REFERRING PRACTICE: PRAC-WLW-129
PRIORITY: ROUTINE
PATIENT: TEST-142670

As per phone call.
```

Referral #159

```
REFERRING PRACTICE: PRAC-HGT-402
PRIORITY: ROUTINE
PATIENT: TEST-304739

66-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #160

```
REFERRING PRACTICE: PRAC-MPL-141
PRIORITY: ROUTINE
PATIENT: TEST-167943

Thank you for seeing this patient.
```

Referral #161

```
REFERRING PRACTICE: PRAC-BRK-786
PRIORITY: ROUTINE
PATIENT: TEST-531655

67-year-old man with poorly controlled asthma: two courses of oral steroids in the
last year. Childhood asthma, worsened after moving to a damp flat. Peak flow 68%
of predicted. Spirometry shows reversible obstruction. Personal asthma action plan
updated; advised 999 if not improving with reliever.
```

Referral #162

```
REFERRING PRACTICE: PRAC-BRK-165
PRIORITY: URGENT
PATIENT: TEST-409521

82-year-old woman with a persistent hoarse voice for four weeks. Current smoker,
20 per day. Drinks around 30 units a week. No neck lumps palpable. Chest X-ray
requested, result awaited. Advised to call 999 if any noisy breathing or
difficulty breathing.
```

Referral #163

```
REFERRING PRACTICE: PRAC-OAK-190
PRIORITY: URGENT
PATIENT: TEST-528627

31-year-old woman with post-menopausal bleeding, two episodes over the past month. Menopause age 51. Not on HRT. BMI 38. Speculum: cervix looks normal. Smear up to date and normal. Told to expect contact within two weeks.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #164

```
REFERRING PRACTICE: PRAC-MPL-482
PRIORITY: ROUTINE
PATIENT: TEST-355211

65-year-old man with a two-year history of worsening bilateral knee pain, now limiting walking to 200 metres. BMI 34. Type 2 diabetes, HbA1c 58. Has completed a 12-week physiotherapy course. Weight-bearing X-ray shows moderate medial compartment narrowing bilaterally. Advised to return if knee becomes hot, swollen or unable to weight-bear.
```

Referral #165

```
REFERRING PRACTICE: PRAC-WLW-557

59-year-old woman with a new breast lump noticed four weeks ago, right upper outer quadrant. Mother diagnosed with breast cancer aged 52. On combined oral contraceptive. Examination: 2 cm firm, non-tender, mobile lump. No axillary lymphadenopathy palpable. Patient aware this is a suspected cancer referral and to expect contact within two weeks.
```

Referral #166

```
REFERRING PRACTICE: PRAC-WLW-259
PRIORITY: ROUTINE
PATIENT: TEST-991953

68-year-old woman with lower back pain radiating to the left leg for 14 weeks. Office worker. Completed physiotherapy; still struggling with sitting. Straight leg raise positive at 40 degrees. Power and reflexes normal. No red flags. Cauda equina safety-netting given verbally and in writing.
```

Referral #167

```
REFERRING PRACTICE: PRAC-CLV-654
PRIORITY: ROUTINE
PATIENT: TEST-341928

71-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #168

```
REFERRING PRACTICE: PRAC-WLW-891
PRIORITY: ROUTINE
PATIENT: TEST-331598

47-year-old woman with poorly controlled asthma: two courses of oral steroids in
the last year. Childhood asthma, worsened after moving to a damp flat. Peak flow
68% of predicted. Spirometry shows reversible obstruction. Personal asthma action
plan updated; advised 999 if not improving with reliever.
```

Referral #169

```
REFERRING PRACTICE: PRAC-WLW-329
PRIORITY: ROUTINE
PATIENT: TEST-255150

40-year-old man with a carpal tunnel picture: numbness of thumb, index and middle
fingers, waking at night. Hypothyroid on levothyroxine. Works on a production
line. Positive Phalen's and Tinel's. No thenar wasting. Steroid injection
declined. To return if weakness or wasting develops.
```

Referral #170

```
REFERRING PRACTICE: PRAC-RVS-931
PRIORITY: ROUTINE
PATIENT: TEST-371949

70-year-old man with lower urinary tract symptoms: frequency, nocturia x3 and poor
stream for a year. Started tamsulosin six months ago with partial benefit. No
haematuria. PSA 2.1 ng/mL. DRE: smooth, moderately enlarged, benign-feeling
prostate. Renal function normal. Advised to attend ED if unable to pass urine.
```

Referral #171

```
REFERRING PRACTICE: PRAC-CLV-546
PRIORITY: ROUTINE
PATIENT: TEST-447016

32-year-old man with gallstones on ultrasound and three episodes of biliary colic
in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #172

```
REFERRING PRACTICE: PRAC-MPL-508
PRIORITY: ROUTINE
PATIENT: TEST-632280

35-year-old woman with a two-year history of worsening bilateral knee pain, now
limiting walking to 200 metres. Osteoarthritis on X-ray 2024. Regular paracetamol
and topical NSAID with limited effect. Weight-bearing X-ray shows moderate medial
compartment narrowing bilaterally. Advised to return if knee becomes hot, swollen
or unable to weight-bear.
```

Referral #173

```
REFERRING PRACTICE: PRAC-RVS-713
PRIORITY: URGENT
PATIENT: TEST-316852

60-year-old woman with a pigmented lesion on the back that has changed shape and
bled over the last two months. Fair skin, multiple sunburns as a child. No
personal history of skin cancer. Dermoscopy: asymmetric, irregular border,
multiple colours, 9 mm. 7-point checklist score 5. Patient informed of suspected
cancer pathway.

CLINICIAN NOTE: Photographs attached to the e-RS referral.
```

Referral #174

```
REFERRING PRACTICE: PRAC-STN-647
PRIORITY: ROUTINE
PATIENT: TEST-272519

54-year-old woman with eight months of fatigue and low mood not responding to
sertraline 100 mg. Second episode of depression. No current suicidal ideation;
protective factors include children. Bloods including TFTs, B12 and ferritin
normal. PHQ-9 18. Crisis line number given; advised to contact 111 option 2 or
attend ED if thoughts of self-harm.
```

Referral #175

```
REFERRING PRACTICE: PRAC-RVS-576

74-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Mirena coil removed last year due to side
effects. Hb 104, ferritin 9 - started on ferrous sulphate. Pelvic ultrasound: 3 cm
intramural fibroid. Advised to seek urgent help if bleeding very heavy with
dizziness.
```

Referral #176

```
REFERRING PRACTICE: PRAC-WLW-198
PRIORITY: ROUTINE
PATIENT: TEST-261295

47-year-old man with chronic plaque psoriasis covering roughly 15% of body surface
area. Failed potent topical steroids and vitamin D analogue over 12 months. DLQI
16. Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek
same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Query psoriatic arthritis; consider joint rheumatology/dermatology
clinic.
```

Referral #177

```
REFERRING PRACTICE: PRAC-CLV-521
PRIORITY: ROUTINE
PATIENT: TEST-832941

49-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #178

```
REFERRING PRACTICE: PRAC-RVS-879
PRIORITY: ROUTINE
PATIENT: TEST-443602

As per phone call.
```

Referral #179

```
REFERRING PRACTICE: PRAC-MPL-936
PRIORITY: URGENT
PATIENT: TEST-731213

66-year-old woman with new-onset atrial fibrillation and symptomatic
breathlessness on mild exertion. Hypertension, previous TIA in 2022. CHA2DS2-VASc
5. ECG confirms AF, rate 118. Started bisoprolol and apixaban. TFTs normal.
Advised to call 999 for chest pain, collapse or worsening breathlessness.
```

Referral #180

```
REFERRING PRACTICE: PRAC-HGT-925
PRIORITY: ROUTINE
PATIENT: TEST-665714

29-year-old man with a carpal tunnel picture: numbness of thumb, index and middle
fingers, waking at night. Symptoms for 9 months. Night splints for 3 months gave
partial relief. Positive Phalen's and Tinel's. No thenar wasting. Steroid
injection declined. To return if weakness or wasting develops.
```

Referral #181

```
REFERRING PRACTICE: PRAC-HGT-838
PRIORITY: ROUTINE
PATIENT: TEST-895589

63-year-old man with recurrent tonsillitis: seven episodes in the past year
requiring antibiotics. Episodes documented in records for the past two years (5
and 6 per year). Currently well between episodes; tonsils grade 2. Advised to seek
urgent review if unable to swallow or one-sided throat swelling.
```

Referral #182

```
REFERRING PRACTICE: PRAC-RVS-106
PRIORITY: ROUTINE
PATIENT: TEST-946243

58-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Psoriasis since teens; new joint pain in fingers over last 6 months.
Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek
same-day review if skin becomes widespread red and hot.
```

Referral #183

```
REFERRING PRACTICE: PRAC-RVS-156

39-year-old woman with persistent cough for six weeks and two episodes of
haemoptysis. Asbestos exposure as a shipyard worker in the 1980s. Chest X-ray: 3
cm right upper lobe opacity. Bloods normal. Advised to attend ED if coughing large
volumes of blood.
```

Referral #184

```
REFERRING PRACTICE: PRAC-MPL-374
PRIORITY: URGENT
PATIENT: TEST-907075

53-year-old man with sudden painless loss of vision in the right eye this morning,
now partially recovered. Hypertension, type 2 diabetes, smoker. Visual acuity 6/12
right, 6/6 left. ECG sinus rhythm. Started aspirin 300 mg. Advised 999 if any
weakness, speech disturbance or further visual loss.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #185

```
REFERRING PRACTICE: PRAC-RVS-839
PRIORITY: URGENT

60-year-old man with a pigmented lesion on the back that has changed shape and
bled over the last two months. Fair skin, multiple sunburns as a child. No
personal history of skin cancer. Dermoscopy: asymmetric, irregular border,
multiple colours, 9 mm. 7-point checklist score 5. Patient informed of suspected
cancer pathway.

CLINICIAN NOTE: Photographs attached to the e-RS referral.
```

Referral #186

```
REFERRING PRACTICE: PRAC-STN-461
PATIENT: TEST-827077

76-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #187

```
REFERRING PRACTICE: PRAC-CLV-659
PRIORITY: URGENT
PATIENT: TEST-215411

32-year-old woman with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #188

```
REFERRING PRACTICE: PRAC-CLV-855
PRIORITY: ROUTINE
PATIENT: TEST-155473

Please assess and advise. Many thanks.
```

Referral #189

```
REFERRING PRACTICE: PRAC-MPL-493
PRIORITY: ROUTINE
PATIENT: TEST-923712

69-year-old woman with lower back pain radiating to the left leg for 14 weeks.
Office worker. Completed physiotherapy; still struggling with sitting. Straight
leg raise positive at 40 degrees. Power and reflexes normal. No red flags. Cauda
equina safety-netting given verbally and in writing.
```

Referral #190

```
REFERRING PRACTICE: PRAC-BRK-502

49-year-old woman with gallstones on ultrasound and three episodes of biliary
colic in four months. Pain after fatty meals, lasting 2-3 hours. No jaundice. LFTs
normal between episodes. Ultrasound: multiple gallstones, CBD 5 mm. Advised to
attend ED if fever, jaundice or pain lasting over 6 hours.
```

Referral #191

```
REFERRING PRACTICE: PRAC-OAK-860
PRIORITY: ROUTINE
PATIENT: TEST-989543

53-year-old man with lower back pain radiating to the left leg for 14 weeks.
Office worker. Completed physiotherapy; still struggling with sitting. Straight
leg raise positive at 40 degrees. Power and reflexes normal. No red flags. Cauda
equina safety-netting given verbally and in writing.
```

Referral #192

```
REFERRING PRACTICE: PRAC-MPL-803
PRIORITY: URGENT

40-year-old man with a persistent hoarse voice for four weeks. Ex-smoker. Works as a teacher, voice use heavy. No neck lumps palpable. Chest X-ray requested, result awaited. Advised to call 999 if any noisy breathing or difficulty breathing.
```

Referral #193

```
REFERRING PRACTICE: PRAC-BRK-889
PRIORITY: ROUTINE
PATIENT: TEST-319195

47-year-old woman with eight months of fatigue and low mood not responding to sertraline 100 mg. Previous trial of citalopram ineffective. Engaged with talking therapies, completed course. Bloods including TFTs, B12 and ferritin normal. PHQ-9 18. Crisis line number given; advised to contact 111 option 2 or attend ED if thoughts of self-harm.

CLINICIAN NOTE: Patient requests a female clinician where possible.
```

Referral #194

```
REFERRING PRACTICE: PRAC-WLW-749
PRIORITY: URGENT
PATIENT: TEST-510027

40-year-old man with a hard, painless left testicular swelling noticed three weeks ago. No trauma. Previous orchidopexy for undescended testis as a child. Examination: firm irregular mass within the body of the testis, separate from epididymis. Patient informed of suspected cancer referral.
```

Referral #195

```
REFERRING PRACTICE: PRAC-CLV-909
PRIORITY: ROUTINE
PATIENT: TEST-328981

75-year-old woman with recurrent tonsillitis: seven episodes in the past year requiring antibiotics. Missing significant time from work. No quinsy. Currently well between episodes; tonsils grade 2. Advised to seek urgent review if unable to swallow or one-sided throat swelling.
```

Referral #196

```
REFERRING PRACTICE: PRAC-MPL-939
PRIORITY: URGENT
PATIENT: TEST-196430

41-year-old man with dysphagia to solids, progressive over six weeks, food
sticking at mid-chest. Long-standing reflux. Lost 4 kg. FBC shows mild microcytic
anaemia, Hb 112. Advised to attend ED if unable to swallow fluids or saliva.
```

Referral #197

```
REFERRING PRACTICE: PRAC-MPL-717
PATIENT: TEST-537554

71-year-old woman with suspected obstructive sleep apnoea: loud snoring, witnessed
apnoeas and daytime sleepiness. BMI 36. HGV driver - advised about DVLA
obligations. STOP-BANG 6. Thyroid function normal. Advised not to drive if feeling
sleepy.

CLINICIAN NOTE: Occupational driver; would appreciate expedited assessment.
```

Referral #198

```
REFERRING PRACTICE: PRAC-RVS-431
PRIORITY: ROUTINE
PATIENT: TEST-491908

80-year-old woman with heavy menstrual bleeding for over a year, flooding and
changing pads hourly on heavy days. Tried tranexamic acid and mefenamic acid;
declined hormonal options. Para 2. Hb 104, ferritin 9 - started on ferrous
sulphate. Pelvic ultrasound: 3 cm intramural fibroid. Advised to seek urgent help
if bleeding very heavy with dizziness.
```

Referral #199

```
REFERRING PRACTICE: PRAC-WLW-625
PRIORITY: ROUTINE
PATIENT: TEST-730469

61-year-old woman with chronic plaque psoriasis covering roughly 15% of body
surface area. Psoriasis since teens; new joint pain in fingers over last 6 months.
Bloods: LFTs and FBC normal. Hep B/C and HIV screen negative. Advised to seek
same-day review if skin becomes widespread red and hot.

CLINICIAN NOTE: Query psoriatic arthritis; consider joint rheumatology/dermatology
clinic.
```

Referral #200

```
REFERRING PRACTICE: PRAC-CLV-880
PRIORITY: ROUTINE
PATIENT: TEST-256588

34-year-old man with gradually worsening hearing in the left ear over a year, with
intermittent tinnitus. Works in construction, inconsistent hearing protection.
Otoscopy normal both sides. Rinne positive bilaterally, Weber lateralises to the
right. To return urgently if sudden further loss of hearing or facial weakness.

CLINICIAN NOTE: Asymmetric loss; please consider MRI IAMs.
```

Answer Key

Complete: 143, Body has no history to work from: 20, Missing PRIORITY line: 11, Missing patient identifier: 13, Missing PRIORITY line and patient identifier: 13

#	Status
001	Complete
002	Body has no history to work from
003	Complete
004	Complete
005	Complete
006	Body has no history to work from
007	Body has no history to work from
008	Body has no history to work from
009	Complete
010	Complete
011	Complete
012	Complete
013	Complete
014	Complete
015	Complete
016	Missing PRIORITY line
017	Missing PRIORITY line
018	Complete
019	Complete
020	Complete
021	Complete
022	Missing patient identifier
023	Complete
024	Missing PRIORITY line
025	Complete
026	Complete
027	Complete
028	Complete
029	Missing PRIORITY line and patient identifier
030	Complete
031	Complete
032	Body has no history to work from
033	Missing patient identifier
034	Complete
035	Complete
036	Complete
037	Complete
038	Complete
039	Missing PRIORITY line
040	Complete

#	Status
041	Complete
042	Complete
043	Complete
044	Body has no history to work from
045	Missing PRIORITY line and patient identifier
046	Complete
047	Complete
048	Missing patient identifier
049	Complete
050	Complete
051	Missing PRIORITY line
052	Complete
053	Body has no history to work from
054	Body has no history to work from
055	Missing patient identifier
056	Complete
057	Complete
058	Complete
059	Complete
060	Complete
061	Complete
062	Complete
063	Complete
064	Complete
065	Complete
066	Complete
067	Missing PRIORITY line
068	Body has no history to work from
069	Complete
070	Missing patient identifier
071	Complete
072	Complete
073	Complete
074	Complete
075	Complete
076	Complete
077	Complete
078	Body has no history to work from
079	Complete
080	Complete
081	Complete
082	Missing PRIORITY line and patient identifier
083	Complete
084	Complete

#	Status
085	Complete
086	Complete
087	Complete
088	Complete
089	Complete
090	Complete
091	Complete
092	Complete
093	Complete
094	Missing PRIORITY line and patient identifier
095	Complete
096	Complete
097	Complete
098	Complete
099	Complete
100	Complete
101	Missing patient identifier
102	Complete
103	Complete
104	Complete
105	Complete
106	Complete
107	Complete
108	Missing patient identifier
109	Complete
110	Complete
111	Missing PRIORITY line and patient identifier
112	Complete
113	Complete
114	Complete
115	Complete
116	Body has no history to work from
117	Missing PRIORITY line and patient identifier
118	Body has no history to work from
119	Complete
120	Complete
121	Complete
122	Complete
123	Missing PRIORITY line and patient identifier
124	Complete
125	Complete
126	Complete
127	Body has no history to work from
128	Complete

#	Status
129	Complete
130	Complete
131	Missing PRIORITY line and patient identifier
132	Complete
133	Body has no history to work from
134	Complete
135	Missing PRIORITY line and patient identifier
136	Missing patient identifier
137	Complete
138	Missing PRIORITY line
139	Complete
140	Complete
141	Body has no history to work from
142	Complete
143	Complete
144	Complete
145	Complete
146	Body has no history to work from
147	Complete
148	Complete
149	Complete
150	Complete
151	Complete
152	Missing patient identifier
153	Missing patient identifier
154	Missing patient identifier
155	Missing PRIORITY line
156	Complete
157	Missing PRIORITY line
158	Body has no history to work from
159	Complete
160	Body has no history to work from
161	Complete
162	Complete
163	Complete
164	Complete
165	Missing PRIORITY line and patient identifier
166	Complete
167	Complete
168	Complete
169	Complete
170	Complete
171	Complete
172	Complete

#	Status
173	Complete
174	Complete
175	Missing PRIORITY line and patient identifier
176	Complete
177	Complete
178	Body has no history to work from
179	Complete
180	Complete
181	Complete
182	Complete
183	Missing PRIORITY line and patient identifier
184	Complete
185	Missing patient identifier
186	Missing PRIORITY line
187	Complete
188	Body has no history to work from
189	Complete
190	Missing PRIORITY line and patient identifier
191	Complete
192	Missing patient identifier
193	Complete
194	Complete
195	Complete
196	Complete
197	Missing PRIORITY line
198	Complete
199	Complete
200	Complete